

WHAT DISABILITY ACTUALLY MEANS

A Plain Language Guide

For People With Disabilities and the People Who Support Them



This guide is for everyone.

If you have a disability — this explains what disability actually *is*, in simple words.

If you support someone with a disability — this gives you the words to explain it.

If you have never thought of yourself as disabled — this is for you too. **Disability does not discriminate.**

Every section has a **green box** at the bottom. That green box gives you exact words to use.

The Alberta Disability System Breakdown — May 2026

PAGE 2 — IT TAKES ONE ACCIDENT. ONE DIAGNOSIS.



Disability is not a club other people belong to.

Disability is a category any human body or brain can enter at any moment. Through illness. Through injury. Through age. Through infection. Through chance.

Most of us will be in this category by the end of our lives — whether we plan for it or not.

How people become disabled:

- A car accident at age 25 can produce permanent brain injury.
- A stroke at age 48 can produce paralysis and language loss.
- A virus can damage the heart, the lungs, or the nervous system — and never go away.
- A fall on ice can produce a spinal cord injury.
- A genetic condition that was hidden for decades can express at age 45.
- Years of repetitive work can produce permanent damage to joints, back, or hearing.
- Aging itself produces cognitive decline, sensory loss, and mobility limits.

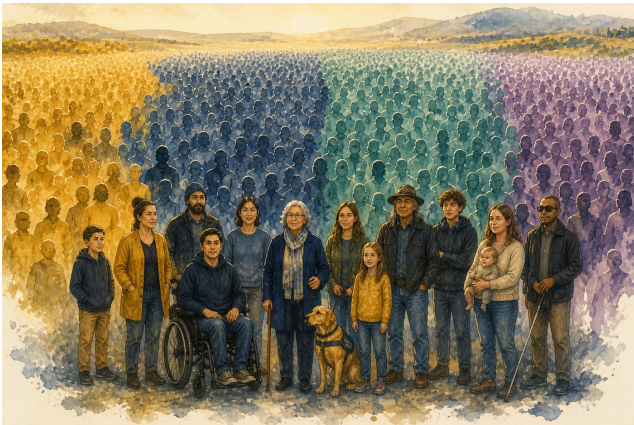
Did You Know?

Disability is not a fixed identity that belongs to other people. It is a category most humans pass into during a normal lifetime. The person who is well at thirty may have a condition all along that becomes apparent at forty-five. The 79,290 Albertans on AISH right now are not a different species. They are people who lived almost identically to you — until something happened.

HOW TO EXPLAIN THIS

Try saying this: "Disability is not something that only happens to other people. It is something that happens to humans. One in five Canadians lives with a disability right now. Most of us will join them eventually — through age, accident, or illness. Public policy on disability is policy about all of us, sooner or later."

PAGE 3 — WHO IS ACTUALLY ON AISH



There are 79,290 Albertans on AISH right now. They have been formally assessed by Alberta's own medical and adjudication system, and approved as having a permanent disability that prevents them from earning a livelihood.

Here is what their primary medical conditions actually are:

Primary Condition	People	Share
Cognitive disorders (autism, intellectual disability, brain injury)	22,419	28.3%
Mental illness (depression, anxiety, schizophrenia, PTSD)	23,940	30.2%
Physical disability (mobility, sensory, chronic illness)	~24,800	~31.3%
Other / multiple primary diagnoses	~8,131	~10.2%
TOTAL	79,290	100%

More than **58%** of AISH recipients — **46,359 people** — have a primary diagnosis of cognitive disorder or mental illness.

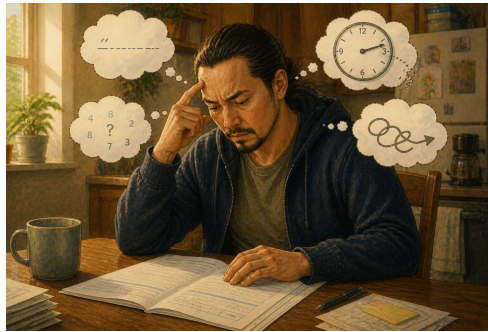
These are conditions whose defining feature is difficulty with sustained focus, complex written language, multi-step administrative tasks, and navigating institutions without support.

The government accepted these people into AISH *because* of those limitations. The same limitations are then ignored every time the program asks them to fill in a form, attend a phone-tree call, or meet a deadline.

HOW TO EXPLAIN THIS

Try saying this: "The government already decided these people have permanent disabilities. They were assessed by Alberta's own doctors and Alberta's own system. The disability was the reason they qualified. Now the same government wants to test them again — using the same disability as a barrier to passing the test."

PAGE 4 — WHAT COGNITIVE DISABILITY MEANS



22,419 Albertans on AISH have a cognitive disability as their primary condition.

Cognitive disability covers conditions that affect how the brain processes information. It includes:

- Intellectual disability (lifelong, present from birth)
- Autism spectrum disorder
- Acquired brain injury (a fall, a car accident, a stroke)
- Dementia and Alzheimer's disease
- Severe ADHD and learning disabilities (dyslexia, dyscalculia)
- Severe mental illness with cognitive components

What cognitive disability can hinder:

Working memory — holding information in mind long enough to use it. The person may forget what they were just told.

Executive function — planning, sequencing, starting, finishing. Opening mail, paying a bill on time, getting to an appointment on the right day.

Reading and comprehension — understanding written language, especially formal or legal language. May need to read something three or four times.

Sustained attention — staying focused for extended periods. Phone trees, multi-page forms, and long appointments all become barriers.

Did You Know?

Most cognitive disabilities are **invisible**. The person standing in line at the grocery store, the person in the office cubicle, the person on the bus — you cannot tell from looking at them. Many people with cognitive disability never tell anyone, because of stigma or fear of losing a job. Invisibility does not mean the disability is mild. It means the disability is hidden.

HOW TO EXPLAIN THIS

Try saying this: "Cognitive disability is not a single thing. It is dozens of conditions that affect memory, planning, reading, and attention. The person you are talking to may have one. They may not have told you. The forms the government sends are written for people whose brains do not work this way."

PAGE 5 — WHAT MENTAL ILLNESS MEANS



23,940 Albertans on AISH have mental illness as their primary condition.

Mental illness on AISH does not mean a passing bad mood. It means severe, chronic, and impairing conditions that have been documented by clinicians, have resisted standard treatment, and have made earning a living impossible.

What this category includes:

- Severe depression that prevents daily function
- Bipolar disorder with manic and depressive episodes
- Schizophrenia and other psychotic disorders
- PTSD and complex trauma (C-PTSD)
- Severe anxiety and panic disorders
- Borderline personality disorder and other personality disorders
- Severe eating disorders (anorexia has the highest mortality rate of any mental illness)

What severe mental illness can hinder:

Concentration — major depression and active psychosis interfere with the brain's ability to focus, plan, remember, and decide. The cognitive impairment is real, and on testing, often indistinguishable from mild dementia.

Sleep — almost all severe mental illness disrupts sleep, which then compounds every other symptom.

Energy and motivation — depression can make leaving the house impossible. Not because the person doesn't want to. Because the brain cannot generate the energy required.

Reality testing — in psychotic conditions, the basic distinction between internal experience and external reality can break down.

Did You Know?

Mental illness is a **leading cause of disability worldwide**. The Institute for Health Metrics and Evaluation found that mental disorders account for 10.6% of all disability-adjusted life years in Canada. The system that treats them is fragmented and underfunded — Alberta's mental health budget is 5.5% of total health spending. The Mental Health Commission of Canada recommends 12%.

HOW TO EXPLAIN THIS

Try saying this: "Severe mental illness is not weakness. It is brain disease. It can be as disabling as a physical injury and harder to recover from because the public system does not pay for the treatment that works. Sliding scale therapy is not a solution when sliding scale starts at eighty dollars a session and AISH is below the poverty line."

PAGE 6 — WHAT PHYSICAL DISABILITY MEANS



Roughly 24,800 Albertans on AISH have a physical disability as their primary condition.

Physical disability is the category most people picture when they hear the word "disability" — but the picture is usually wrong. Most physical disability is not a person in a wheelchair. Most physical disability is invisible from outside.

What this category includes:

Mobility conditions — paralysis, spinal cord injury, cerebral palsy, severe arthritis, muscular dystrophy, multiple sclerosis. Some require wheelchairs. Many do not.

Sensory conditions — blindness, low vision, deafness, hard-of-hearing, deaf-blindness.

Chronic pain conditions — fibromyalgia, severe chronic back pain, complex regional pain syndrome, arthritis, endometriosis. Approximately 8 million Canadians live with chronic pain.

Chronic illness — Crohn's disease, lupus, ME/CFS, long COVID, severe asthma, kidney failure, heart disease.

Episodic conditions — conditions that fluctuate. Good days followed by weeks where leaving the house is impossible. The disability is not the bad day. The disability is the unpredictability.

Did You Know?

About 1 in 2 Canadians will be diagnosed with cancer in their lifetime. Many will recover. Some will live with permanent disability afterward — from the cancer, the chemotherapy, the radiation, or the surgery. The cancer survivor with permanent neuropathy or fatigue is a person with a physical disability even though the cancer itself is gone.

HOW TO EXPLAIN THIS

Try saying this: "Physical disability is more than mobility. It is pain, fatigue, chronic illness, and conditions that fluctuate. The person who looks fine today may be unable to leave the house tomorrow. That is not laziness. That is the disability — the unpredictability is what makes consistent employment impossible."

PAGE 7 — INTELLECTUAL AND DEVELOPMENTAL DISABILITIES



Intellectual disability is a specific type of cognitive disability. It is lifelong. It is present from birth or early childhood.

Intellectual disability does not improve with medication or treatment. It can range from mild — capable of independent living with some support — to profound — requiring total care for daily living tasks.

In Alberta, an IQ of 70 or below combined with significant adaptive skill limitations beginning before age 18 is the threshold for the **Persons with Developmental Disabilities (PDD)** program.

Many adults with intellectual disability receive both AISH (income) and PDD (services). When PDD intake gets longer, when caseworkers leave, when programs close — those adults lose support. Their intellectual disability does not lessen. The supports that helped them live with it disappear.

Autism spectrum disorder is a different category. Autism is a neurodevelopmental condition affecting how a person communicates, processes social information, and experiences sensory input. It is a spectrum: support needs vary widely. Some autistic adults live independently. Others require continuous care. Many have significant strengths — pattern recognition, deep focus, exceptional memory in areas of interest — alongside their challenges.

Autism is not an illness. It is a different way the brain is wired, present from birth, lifelong.

Did You Know?

Adults with intellectual disability and autism have **higher rates of anxiety, depression, and other mental health conditions** than the general population — and far less access to appropriate mental health treatment. Many psychologists and therapists do not have training in supporting clients with intellectual or developmental disability. The result is two disabilities, often layered, with treatment for neither.

HOW TO EXPLAIN THIS

Try saying this: "Intellectual disability is not a phase or a result of bad teaching. It is a lifelong condition that begins before age 18. The person with intellectual disability needs lifelong support — and AISH plus PDD is the system Alberta built to provide it. Cutting either one is not reform. It is removing the supports that make their lives possible."

PAGE 8 — CONDITIONS PRESENT FROM BIRTH



About 3% of all live births in Canada involve some form of congenital condition.

Some are minor and produce no lasting disability. Others produce substantial lifelong disability. Common conditions in the AISH population include:

Cerebral palsy — a group of disorders affecting movement and muscle tone, caused by damage to the developing brain. Severity ranges from mild (slight movement difference) to severe (full wheelchair use, communication device, complex daily care).

Down syndrome — a genetic condition producing intellectual disability and characteristic physical features. People with Down syndrome live full lives with appropriate support, and life expectancy has substantially increased over recent decades.

Spina bifida — a neural tube defect where the spine does not close completely during fetal development. Severe forms produce paralysis, hydrocephalus, and bowel and bladder dysfunction.

Cystic fibrosis — a genetic condition affecting lungs and digestive system. Treatment is intensive, lifelong, and includes hours of daily medical management.

Muscular dystrophy — genetic conditions causing progressive muscle weakness. Some forms produce disability through adolescence and adulthood.

Congenital heart defects — structural problems with the heart present at birth. Severe forms limit physical capacity and require multiple surgeries across the lifespan.

Did You Know?

Some genetic conditions express in childhood. Others do not express until adulthood. **Huntington's disease** typically becomes apparent in middle age. Some forms of **muscular dystrophy**, **hereditary cancers**, and **early-onset dementia** do not announce themselves until forty or fifty. The person who is well at thirty may have been carrying a condition all along.

HOW TO EXPLAIN THIS

Try saying this: "Congenital conditions are not chosen and not preventable. The person was born with them. They are part of the population AISH was designed to support. The fact that some genetic conditions do not show up until adulthood is exactly why disability can happen to anyone — including people who feel completely healthy right now."

PAGE 9 — THE "ONE EVENT" CATEGORY



Acquired conditions are conditions a person did not have, then had — often through a single event.

These are the disabilities that prove disability is not a fixed identity. They are entered through one moment.

Traumatic brain injury — Brain Injury Canada estimates approximately 165,000 Canadians sustain a brain injury each year. Most recover. Tens of thousands are left with permanent disability. A single car accident at age twenty-five can produce a brain that, at age thirty, cannot reliably hold information, regulate emotions, or sustain attention.

Stroke — A stroke at age forty-eight can produce paralysis on one side of the body, language loss, vision problems, memory loss, and fatigue. None of those existed twenty-four hours before the event.

Spinal cord injury — A fall, a car accident, a sports injury can sever or damage the spinal cord. Depending on level of injury, the person may experience paraplegia, tetraplegia, or partial impairment.

Long COVID and post-viral conditions — A single viral infection can produce a condition that never fully goes away. Long COVID has produced disability in healthy adults across every age range. Post-Lyme, post-mono, and post-sepsis conditions have done the same for decades.

Workplace injury — Decades of occupational exposure to repetitive strain, chemicals, noise, vibration, and physical stress produce disability that often does not appear until later in a working life.

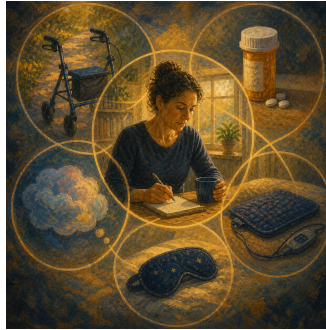
Did You Know?

Disability is not, as a rule, the condition of people who lived differently from you. It is, much more often, the condition of people who lived almost identically to you, until something happened. The something might be a fall on ice. A car crash. A diagnosis after a routine appointment. A virus you didn't even know you had.

HOW TO EXPLAIN THIS

Try saying this: "It takes one accident. One diagnosis. One virus. The 79,290 people on AISH right now were, on average, no different from anyone else — until something happened. The disability safety net is for them. It is also for you, eventually, in a form none of us can predict."

PAGE 10 — DISABILITY RARELY ARRIVES ALONE



Most people on AISH do not have one condition. They have several.

The classification "primary medical condition" is administrative shorthand. It means: this was the lead clinical basis for AISH approval. It does *not* mean the person has only that condition.

Common combinations:

Mental illness with chronic pain. Estimates of depression among people with chronic pain range from 31% to 100%. The two amplify each other.

Brain injury with mental illness. Brain injury substantially increases the risk of subsequent depression, anxiety, PTSD, and substance use disorder. The injury itself produces emotional dysregulation that compounds mental health vulnerability.

Autoimmune disease with cognitive impairment. Lupus, multiple sclerosis, and other autoimmune conditions frequently produce cognitive symptoms — sometimes called "lupus fog" or "MS fog" — alongside their physical manifestations.

Chronic illness with mental illness. Living with painful or disabling physical illness substantially elevates the risk of secondary depression and anxiety. The mental illness is not separate from the physical condition. It is a predictable consequence of it.

Developmental disability with mental illness. Adults with intellectual disability and autism have higher rates of anxiety and depression — and far less access to appropriate mental health treatment.

Did You Know?

Public assumptions about disability often imagine a single condition — "the person with the wheelchair," "the person with depression." The clinical reality is closer to: a person whose original diagnosis has produced a cascade of secondary conditions, each interacting with the others, all of which require management at the same time. **The supports designed for any one condition almost never address the whole picture.**

HOW TO EXPLAIN THIS

Try saying this: "When you cut a service for one disability, you do not just affect people with that one condition. You affect people with three, four, five overlapping conditions who depended on that service to manage everything else. Disability stacks. The supports have to stack too — or the whole structure falls."

PAGE 11 — VISIBLE AND INVISIBLE DISABILITY



Most disability is invisible.

If you imagine a disabled person, you probably picture a wheelchair, a white cane, a hearing aid, or a service dog. Those are real. They are also the minority.

Conditions that are usually invisible to outside observers:

- Severe depression, bipolar disorder, schizophrenia, PTSD, anxiety
- Chronic pain, fibromyalgia, endometriosis
- Autoimmune conditions (lupus, MS, Crohn's, rheumatoid arthritis)
- ME/CFS and long COVID
- Acquired brain injury, mild and moderate
- Most of autism
- ADHD and learning disabilities
- Cognitive effects of medication (chemotherapy, psychiatric drugs, opioids)
- Fatigue, brain fog, post-exertional malaise

The person who "looks fine" is not faking. They have done the work to get out the door. What you see on the bus or in the grocery store is the result of hours of preparation that the disability made expensive. They may collapse for two days afterward.

Stigma and fear of losing benefits or jobs cause many people to hide their conditions. Invisibility is often a survival strategy — not a sign that the disability is mild.

Did You Know?

Public sympathy follows visibility. When a person uses a wheelchair, the disability is acknowledged. When the same person, on a different day, walks the short distance to the mailbox using their cane, they are often accused of **faking it**. The chair did not lie. The cane is not lying either. Disability fluctuates. Mobility aids are tools — and people use the tool that fits the day.

HOW TO EXPLAIN THIS

Try saying this: "You cannot tell who is disabled by looking. The person who looks fine in line at the coffee shop may have run out of energy by the time they get home. Disability is real even when you cannot see it. Believing people about their own conditions is the starting point — not something they have to prove to you first."

PAGE 12 — WHAT AISH IS, AND WHAT IT IS NOT



AISH is not welfare. AISH is not unemployment.

AISH stands for **Assured Income for the Severely Handicapped**. It is Alberta's program for adults with permanent disabilities that prevent them from earning a livelihood.

To qualify, a person must satisfy **all** of these criteria:

- **Permanent disability.** The medical condition must be permanent. Not a phase. Not temporary. Not expected to improve.
- **Substantial limitation on earning a livelihood.** The disability must substantially limit the person's ability to earn a living through work.
- **Physician medical report.** A doctor must complete a 9-page Part B Medical Report.
- **Financial criteria.** Income, assets, and family circumstances are reviewed.

Approval requires going through a formal adjudication process — not just declaring a disability. Alberta's own contracted medical professionals review the application against the government's own criteria.

Once approved, the finding is a **governmental determination**. It is not provisional. It does not expire automatically. The government has formally found that this person has a permanent disability preventing them from earning a living.

Did You Know?

The current AISH benefit is **\$1,901/month maximum**. The Maytree Foundation calculates this is **\$6,004 below Canada's official poverty line annually**. AISH was already inadequate before any of the recent changes. The base benefit, which has not been indexed to inflation in years, is what 79,290 disabled Albertans live on.

HOW TO EXPLAIN THIS

Try saying this: "AISH is the income program for people the government has formally found to have permanent disabilities preventing them from working. It is below the poverty line. The recipients are not avoiding work — the government already certified that work is not possible for them."

PAGE 13 — WHAT ADAP DOES



On July 1, 2026, the Alberta Disability Assistance Program (ADAP) begins.

All 79,290 current AISH recipients are automatically transferred to ADAP. To stay on AISH, each person must reapply through a new **Medical Review Panel**, which has the final say. The panel's decisions cannot be appealed to the Citizens Appeal Panel — that independent appeal body was eliminated by **Bill 12** in December 2025.

What is being reversed:

Every current AISH recipient was assessed and approved by Alberta's own process, using its own medical criteria, by its own contracted medical professionals. The government looked at each of these 79,290 people and made a formal finding: **this person has a permanent disability that prevents them from earning a living.**

ADAP reverses that finding — for all 79,290 people simultaneously. Without new medical evidence. Without individual reassessment. Without published criteria explaining why the original finding is no longer valid. Without any independent way to challenge the reversal.

The presumption is reversed:

Under AISH, a person approved is assumed to remain eligible unless evidence indicates otherwise. Under ADAP, the reverse applies: every recipient defaults to the lower-benefit program and must actively prove their continued eligibility — with no appeal. This is the burden of proof, applied to people with permanent disabilities, who were already assessed and approved.

Did You Know?

Government statute requires AISH eligibility to be based on a permanent disability that substantially limits ability to earn a livelihood. ADAP's framing — that recipients can simply be employed — contradicts the law that defines who AISH was for in the first place. **83.6% of AISH recipients had zero employment income in 2023-24.** Not because AISH discourages work. Because the disabilities the government qualified them on make sustained employment impossible.

HOW TO EXPLAIN THIS

Try saying this: "The same government that approved 79,290 people for AISH is now reversing all 79,290 of those approvals at once — without new evidence, without reassessing anyone individually, and without any independent appeal. Bill 12 took away the appeal panel. The disability didn't change. The system did."

PAGE 14 — THE STRUCTURAL CONTRADICTION



The same condition that qualifies a person for the program disqualifies them from navigating it.

The Government of Alberta determined that 79,290 people have severe and permanent disabilities that prevent them from earning a livelihood.

The same Government of Alberta then designed a compliance system that requires those same 79,290 people to:

- Read forms written at Grade 12 to 16 reading level
- Complete multi-step paperwork chains across multiple agencies
- Find a family doctor in a province where 650,000 people don't have one
- Pay physician fees out of pocket on a fixed income below the poverty line
- File annual taxes with no refund and no taxable income
- Meet deadlines with no flexibility for medical flares or hospitalizations
- Navigate phone trees, online portals, and appeal processes — all unaccommodated

The contradiction: The defining clinical features of the largest AISH populations — cognitive disorders, mental illness — include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step administrative tasks, and difficulty navigating institutional systems without external support.

These are not aspersions. They are the diagnostic criteria. They are the basis on which the government approved them in the first place.

Did You Know?

The recipient population was assessed and accepted into AISH because of these limitations. The same limitations are then ignored every time the program asks them to perform administrative work that many adults without disabilities find difficult. The system penalizes people for the very disability that qualified them.

HOW TO EXPLAIN THIS

Try saying this: "The government accepted these people into AISH because they cannot do this kind of administrative work. Then the government built a system that requires them to do this kind of administrative work. That isn't reform. That's a system designed to lose people."

PAGE 15 — WHAT YOU CAN DO



This is a public education document. Public ignorance is what enables public indifference.

Education is the precondition for change. The more people know what disability actually is — and what the 79,290 Albertans on AISH are actually living with — the harder it becomes to dismiss them.

If you have a disability:

- Use the green boxes in this guide. They are written for you to share, post, or read aloud.
- Share this document with one person who does not understand your situation.
- You are not alone. There are 79,289 other Albertans in this same fight right now.

If you support someone with a disability:

- Read this guide and share it with people who ask why "the government is making changes."
- Use the exact words in the green boxes when you encounter the assumptions this guide is meant to correct.
- Help the person you support file paperwork, attend appointments, and document everything.

If this is the first you've heard of any of this:

- Send this document to one other person.
- Read the long version: **What Disability Actually Means — Reference Document** (36 pages, fully sourced).
- Visit albertadisabilitysystembreakdown.netlify.app for the full archive.
- Contact your MLA. Tell them you read this. Ask them what their position is.

Did You Know?

You will be in this category, eventually. One in five Canadians lives with a disability right now. By the time you reach old age, that number will likely be one in two. The disability income system you build today is the disability income system you may need tomorrow.

HOW TO EXPLAIN THIS

Try saying this: "Disability does not discriminate. The system you allow to be cut today is the system you may depend on tomorrow. The 79,290 Albertans on AISH right now are not asking for sympathy — they are asking for the program the government already approved them for to keep working. That is not a high bar. That is the floor."

WHAT DISABILITY ACTUALLY MEANS



It takes one accident.

One diagnosis.

Disability does not discriminate. It is not a moral failure. It is not a personal weakness. It is not a fixed identity that belongs to other people.

It is a category any human body or brain can enter at any moment — through illness, injury, age, infection, or chance — and it is a category most of us will be in by the end of our lives whether we plan for it or not.

Public ignorance about what disability actually is enables public indifference to what disabled people actually need.

Education is the precondition for change.

Companion document to: *What Disability Actually Means — A Reference-Grade Public Education Document on the Conditions, Lives, and Realities Behind the Numbers* (36 pages, fully sourced).

Available with the full ADSB document archive at:
albertadisabilitysystembreakdown.netlify.app

The Alberta Disability System Breakdown — Advocate

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